

SUSPECT ADVERSE REACTION REPORT

CIOMS FORM I : CASE 1

I. REACTION INFORMATION

1. PATIENT INITIALS (first, last) Not Reported	1a. COUNTRY India	2. DATE OF BIRTH Not Reported	2a. AGE 22 Years
3. SEX Male	4-6. REACTION ONSET UNKNOWN- relative chronology used (3 days after initiation)		
8-12. CHECK ALL APPROPRIATE TO ADVERSE REACTION ☐ PATIENT DIED ☒ INVOLVED OR PROLONGED INPATIENT HOSPITALISATION ☐ INVOLVED PERSISTENCE OR SIGNIFICANT DISABILITY OR INCAPACITY ☒ LIFE THREATENING			
7 + 13. DESCRIBE REACTION(S) (including relevant tests/lab data) Development of severe skin manifestations, including extensive skin sloughing, a burning sensation across the body, and small blisters distributed across the trunk and all four limbs. Physical examination revealed widespread erythematous macular lesions displaying darker hemorrhagic patterns, along with active dislodgement of the epidermis on the limbs. There was no mucosal or genital involvement noted. Nikolsky's sign was explicitly positive. Based on the clinical presentation and dermatologist evaluation, the patient was diagnosed with amoxicillin-induced Toxic Epidermal Necrolysis (TEN). Lab Values: Hb: 10.5 gm%, TC: 1200 cells/cu mm, RBS: 84 gm/dl, BUN: 20 mg/dl, Bicarbonates: 23 mEq/l, Platelets: 2.0 lakhs, Serum glucose: 110 mg/dl. LFTs and RFTs: Normal. The severity score (SCORTEN) was calculated as 1, based on >10% epidermal detachment, predicting a low mortality risk of 3.2%.			

II. SUSPECT DRUG(S) INFORMATION

14. SUSPECT DRUG(S) (include generic name)	Drug 1: Amoxicillin-clavulanic acid (Suspect: Amoxicillin)	Drug 2: Amoxicillin sodium (Injection)
15. DAILY DOSE(S)	625 mg twice daily	250 mg twice daily

16. ROUTE(S) OF ADMINISTRATION	Oral (Tablet)	Intravenous / Intramuscular
17. INDICATION(S) FOR USE	Fever	Fever
18. THERAPY DATES (from/to)	Unknown RELATIVE CHRONOLOGY USED Start: 3 days prior to admission Stop: Date of admission	Unknown RELATIVE CHRONOLOGY USED Start: 3 days prior to admission Stop: Date of admission
19. THERAPY DURATION	3 days	3 days
20. DID REACTION ABATE AFTER STOPPING DRUG? [X] YES [] NO [] NA	Positive Dechallenge	Positive Dechallenge
21. DID REACTION REAPPEAR AFTER REINTRODUCTION? [] YES [] NO [X] NA	Not performed / Not ethically feasible	Not performed / Not ethically feasible

III. CONCOMITANT DRUG(S) AND HISTORY

22. CONCOMITANT DRUG(S) AND DATES OF ADMINISTRATION (exclude those used to treat reaction)

Clavulanic acid (co-formulated with the suspect tablet Amoxicillin-clavulanic acid, administered twice daily for 3 days prior to admission).

23. OTHER RELEVANT HISTORY (e.g. diagnostics, allergies, pregnancy with last month of period, etc.)

Patient had a history of fever with onset 3 days prior to hospital admission. No other significant medical history, baseline comorbidities, chronic diagnostics, or drug allergies were reported.

IV. MANUFACTURER INFORMATION

24a. NAME AND ADDRESS OF MANUFACTURER Unknown Literature report	24b. MFR CONTROL NO. Unknown Literature report	24c. DATE RECEIVED BY MANUFACTURER Unknown Literature report
24d. REPORT SOURCE [] STUDY [X] LITERATURE [X] HEALTH PROFESSIONAL	DATE OF THIS REPORT 10-Jul-2026	25a. REPORT TYPE [X] INITIAL [] FOLLOWUP

Literature Citation Details:

Authors: Lakshmi Narasimha G., Kalpana P.

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